

ST. JUDE MEDICAL CENTER - DEPARTMENT OF NEUROLOGY & MEMORY DISORDERS

1200 Healthcare Parkway, Building B, Suite 400 • Confidential Medical Record

Patient Name: Kingsley, Harold 'Hal' R.	Sex: Male	MRN: 3391-8840
DOB / Age: 02/18/1942 (Age: 84)	Exam Date: 08/22/2026	Severity: CRITICAL / Severe
Attending Physician: Dr. Sarah Jenkins, MD (Neurology)	Referring Physician: Emergency Department / Adult Protective Services Liaison	Status: Completed

REASON FOR EVALUATION

Urgent neurological consultation for severe cognitive collapse, nocturnal wandering into traffic, combative sundowning agitation, and total functional failure. Accompanying caregiver: Karen Kingsley (Daughter, primary caregiver in acute distress).

CLINICAL HISTORY (History of Present Illness)

The patient is an 84-year-old male with a 6-year history of progressive dementia, brought in for emergency evaluation following a catastrophic functional breakdown over the last 6 months. Caregiver reports patient wandered into oncoming traffic at 3:00 AM barefoot, believing he was reporting to an army post. Patient exhibits severe verbal and physical combativeness during evening hours (severe sundowning agitation), frequently striking caregivers. Patient has lost meaningful conversational speech, communicating in fragmented echolalia and unintelligible vocalizations. Functional Assessment: Completely dependent (100%) in all instrumental activities of daily living (medication, finance, cooking, driving, navigation, housekeeping) and severely dependent in basic ADLs (incontinent of bowel and bladder, requires 2-person physical assistance for feeding, bathing, and transfer). Behavioral Symptoms: Severe unrelenting nocturnal agitation, profound apathy during daylight, complete sleep cycle inversion, pacing, and severe social withdrawal. Visual illusions and paranoid accusations of impostors (Capgras phenomenon) reported.

PAST MEDICAL HISTORY & MEDICATIONS

Past Medical History: Advanced Dementia, Atrial Fibrillation, Coronary Artery Disease s/p CABG, Recurrent Aspiration Pneumonias.

Active Medications: Donepezil 10 mg PO daily, Memantine 10 mg PO BID, Apixaban 5 mg PO BID, Metoprolol Succinate 50 mg PO daily

NEUROLOGICAL & PHYSICAL EXAMINATION

General: Frail, disheveled, severely confused male in moderate distress, actively resisting examination. Cranial Nerves: Grossly intact, blink to threat preserved. Motor: Bilateral paratonia (gegenhalten rigidity), generalized muscle wasting. Reflexes: Primitive grasp and snout reflexes bilaterally positive. Plantar responses extensor on left. Gait: Severe gait apraxia, unable to stand or walk without maximum 2-person assist, wheelchair-bound. High fall risk.

STANDARDIZED COGNITIVE TESTING: Mini-Mental State Examination (MMSE) (Score: 7/30 - Severe Impairment / Late-Stage Dementia)

- **Orientation:** 0/10 (Completely disoriented to year, season, date, day, month, hospital, city)
- **Registration:** 2/3 (Able to repeat only 2 words with multiple trials)
- **Attention Calculation:** 0/5 (Unable to perform spell WORLD backwards or serial 7s)
- **Recall:** 0/3 (Zero spontaneous recall, no benefit from multiple-choice cueing)
- **Language Visuospatial:** 5/9 (Severe expressive anomia, unable to follow 3-stage command, clock drawing completely unrecognizable scribbles)

DIAGNOSTIC WORKUP & BIOMARKERS

- **Laboratory Findings:** Metabolic panel shows mild dehydration (BUN/Cr 28/1.2), Urinalysis negative for acute UTI, TSH normal, B12 normal.
- **Brain MRI Biomarkers:** End-stage bilateral hippocampal atrophy (MTA Score: Grade 4) with severe ex-vacuo ventriculomegaly and diffuse global cortical ribbon thinning. Moderate periventricular ischemic changes (Fazekas Grade 2).

DIAGNOSTIC IMPRESSION

Primary Diagnosis: Severe Major Neurocognitive Disorder due to Advanced Alzheimer's Disease with Severe Behavioral Disturbance (ICD-10: G30.9 / F02.811)

Clinical Summary: 84-year-old male with end-stage Alzheimer's disease presenting with critical behavioral emergency (elopement into traffic, nocturnal combativeness), MMSE 7/30, MTA Grade 4, primitive reflexes, and total ADL/IADL collapse, requiring urgent secure memory care placement.

PLAN & CLINICAL RECOMMENDATIONS

- Immediate emergency respite placement in a secure, locked memory care facility to ensure patient safety and address acute caregiver burnout.
- Low-dose Quetiapine (Seroquel) 12.5 mg PO at bedtime with cautious titration to 25 mg for severe refractory agitation and sundowning aggression.
- Taper Donepezil if poor tolerance; maintain Memantine.
- Hospice and palliative care consultation for end-stage dementia advance directive goals of care.
- Implement high-level fall precautions, bed alarms, and 24-hour direct supervision.

Electronically Signed: Dr. Sarah Jenkins, MD (Neurology)

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